

High-Velocity Low-Amplitude (HVLA) for Dropped Navicular/Cuboid/Cuneiforms (Hiss Whip)

1. The patient is asked to lie prone on the table, with the physician standing at the side of the table closer to the foot of the table and on the side of the somatic dysfunction.
2. The physician flexes the patient's hip and knee on the side of the dysfunction and drops the leg on the side of the dysfunction off the side of the table:



Fig. 11.177

3. The physician uses both hands to grasp the patient's foot, placing their thumbs in a "V" shape over the plantar surface of the cuboid, navicular or cuneiforms, whichever it is that is in a dropped position relative to the other metatarsals.
4. The physician delivers a downward thrust* through their thumbs into the sole of the foot at the level of the dysfunctional cuboid/navicular/cuneiform while simultaneously inducing a whip-like action at the patient's ankle and knee (hence, the name "Hiss Whip" technique).



Fig. 11.178

** The physician can introduce a more medial thrust for the medial tarsals and a more lateral thrust for the lateral tarsals (the lateral aspect of the foot is opened by adducting the forefoot).*

5. The physician reassesses the range of motion of the forefoot and palpates for the dropped cuboid/navicular/cuneiform to determine effectiveness of treatment.

[Technique #10 in LE Region Section \(Video 11.36\) in Nicholas & Nicholas](#)

HVLA Anterior Tibia on the Femur somatic dysfunction

1. The patient is asked to sit on the side of the table with a small pillow beneath the thigh on the side of the dysfunction as a cushion; the physician is seated at the side of the table facing the patient.
2. The physician places their thumbs on the anterior tibial plateau with the fingers wrapping around the leg:



Fig. 11.156

3. The physician ensures the patient's thigh and leg are relaxed by springing the thigh up and down:



Fig. 11.157

4. The physician delivers a thrust straight down toward the floor (traction down), while simultaneously applying a posterior pressure impulse with the thumbs (multidirectional thrust; white arrows in the image below):



Fig. 11.158

5. The physician reassesses anterior glide and the range of motion of the knee to determine effectiveness of treatment.

[Technique #2 In LE Region Section \(Video 11.28\) in Nicholas & Nicholas](#)

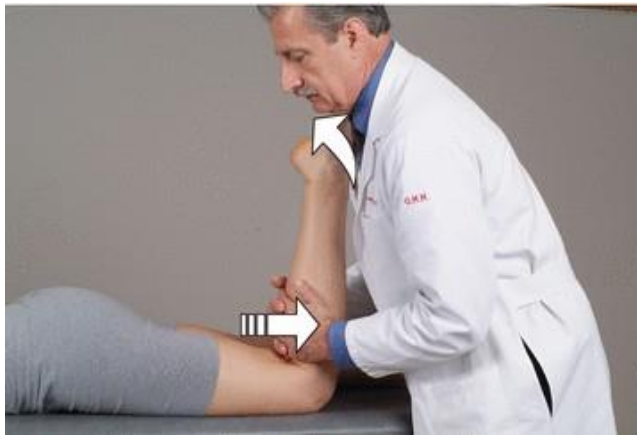
HVLA Posterior Tibia on Femur Somatic Dysfunction

1. The patient is asked to lie prone with the dysfunctional knee flexed to approximately 90 degrees if possible; the physician is standing or sitting at the foot of the table.
2. The dorsum of the patient's foot is placed on the anteromedial aspect of the physician's shoulder (this placement will induce plantar flexion of the foot, taking tension off the gastrocnemius muscle).
3. The physician interlaces and wraps their fingers around the proximal tibia just distal to the popliteal region as shown below:



g. 11.159

4. The physician delivers a thrust with both hands toward themselves and parallel to the table as follows:



g. 11.160

5. The physician reassesses posterior glide and the range of motion of the knee to determine effectiveness of treatment.

[Technique #3 in LE Region Section \(Video 11.29\) in Nicholas & Nicholas](#)

HVLA Posterior Dysfunction of the Fibular Head

Knee: Proximal Fibular Dysfunction

Long-Lever, Fulcrum/Eversion Emphasis

Ex: Left Posterior Fibular Head

Technique

1. The patient lies prone with the dysfunctional knee flexed at 90 degrees.
2. The physician stands at the side of the table opposite the side of the dysfunction
3. The physician places the MCP of the cephalad index finger behind the dysfunctional fibular head, and the hypothenar eminence is angled down into the hamstring musculature to form a wedge behind the knee
4. The physician's caudad hand grasps the ankle on the side of dysfunction and gently flexes the knee until the restrictive barrier is reached



Steps 1 to 4 above

5. The patient's foot and leg are gently externally rotated to carry the fibular head back against the fulcrum formed by the physician's cephalad hand (*white arrow below*)



6. The physician's caudad hand, controlling the patient's foot and ankle, delivers a thrust toward the patient's buttock in a manner that would normally result in further flexion of the knee (*white arrow below*). However, the wedge fulcrum formed by the physician's cephalad hand prevents any such motion.